



To be sold by retail on the prescription of an oncologist/ cancer hospital/ institution only

CARBOL

Carboplatin Concentrate for Solution for Infusion 10 mg/ml

A4 PRODUCT DESCRIPTION
CARBOL - Carboplatin Concentrate for Solution for Infusion 10mg/ml is a clear colorless to faintly yellow solution.
After dilution: A clear colorless to faintly yellow solution.
COMPOSITION
Each ml contains:
Carboplatin BP 10 mg
Water for Injections BP q.s.
A5 PHARMACODYNAMICS
Carboplatin, like Cisplatin, interferes with DNA intrastrand and interstrand crosslinks in cells exposed to the drug. DNA reactivity has been correlated with cytotoxicity.
Paediatric patients: safety and efficacy in children have not been established.

A6 PHARMACOKINETICS
Absorption: After a 1-hour infusion (20-520mg/m²), plasma levels of total platinum and free (ultra filterable) platinum decay biphasically following first order kinetics. For free platinum, the initial phase (t₁ alpha) half life is approximately 90 minutes and the later phase (t₁ beta) half life approximately 6 hours. All free platinum is in the form of carboplatin in the first 4 hours after administration.
Distribution: Protein binding of carboplatin reaches 85-86% within 24 hours of administration, although during the first 4 hours, only up to 29% of the dose is protein bound. Patients with poor renal function may require dosage adjustments due to altered pharmacokinetics of carboplatin.
Elimination: Carboplatin is excreted primarily by glomerular filtration in urine, with recovery of 65% of a dose within 24 hours. Most of the drug is excreted within the first 6 hours. Approximately 32% of a given dose of carboplatin is excreted unchanged. Carboplatin clearance has been reported to vary by 3- to 4- fold in paediatric patients. As for adult patients, literature data suggest that renal function may contribute to the variation in carboplatin clearance.

A7 INDICATION
Carboplatin is indicated in the treatment of advanced stage ovarian cancer of epithelial origin.

A8 RECOMMENDED DOSE
Dosage and Administration: Carboplatin should be used by the intravenous route only. Carboplatin is given in doses of 400 mg/m² body surface area by intravenous infusion over 15 minutes to 1 hour in previously untreated adults (with normal renal function).
Doses should not be given more frequently than every 4 weeks, and should be reduced in patients at risk of myelosuppression. Lower doses may also be given as part of combination regimens. Determination of the hematological nadir by weekly blood counts is recommended for adjusting future dose and scheduling of carboplatin therapy.

Renal Impairment: As carboplatin is excreted by the kidney and is nephrotoxic, the optimum dosage should be determined by frequent monitoring of the hematological nadir and renal function.
Safety and effectiveness in pediatric patients have not been established. Clinically significant hearing loss has been reported to occur in pediatric patients when carboplatin was administered at higher than recommended doses in combination with other ototoxic agents. Immediately before use each vial of Carboplatin should be diluted to concentrations as low as 0.5 mg/ml with 5% Dextrose solution or 0.9% Sodium Chloride solution.

Dose Modifications:
1. In patients with renal failure

Creatinine Clearance (mL/min)	Starting Dose (mg/m ²)
>60	no dose reduction
41 - 59	250
16 - 40	200

Creatinine clearance (mL/min) = N(140 - Age) Weight (kg) / Serum creatinine (mol/L)

Where N = 1.04 for females and 1.23 for males
2. Dose of carboplatin has to be reduced
- When it is used along with aminoglycosides as they increase the toxic effects of carboplatin
- In patients having myelosuppression

A9 ROUTE OF ADMINISTRATION
Intravenous

A10 CONTRAINDICATIONS
Carboplatin is contraindicated in:
- Hypersensitivity to the active substance(s) or to any of the excipients
- patients with severe myelosuppression
- patients with pre-existing severe renal impairment (with creatinine clearance of ≤ 30 ml per minute) unless in the judgment of the physician and patient, the possible benefits of treatment outweigh the risks
- patients with bleeding tumors
- concomitant use with yellow fever vaccine
- patients with a history of severe allergic reaction or other platinum containing compounds.
Dosage adjustment may allow use in the presence of mild impairment.

A11 WARNINGS AND PRECAUTIONS
Warnings:
Myelosuppression: Myelosuppression as a result of carboplatin treatment is closely related to the renal clearance of the drug. Therefore, in patients with abnormal renal function, or who are receiving concomitant therapy with nephrotoxic drugs, myelosuppression, especially thrombocytopenia, may be more severe and prolonged.
The occurrence, severity and protraction of toxicity is likely to be greater in patients who have received extensive prior treatment with the drug for their disease or with cisplatin, have poor performance status and are advanced in years. Renal function parameters should be assessed prior to, during and after carboplatin therapy. Initial carboplatin dosages in these groups of patients should be appropriately reduced and the effects carefully monitored through frequent blood counts between courses. Myelosuppressive effects may be additive to those of concomitant chemotherapy.
Peripheral blood counts (including platelets, white blood cells and haemoglobin) should be followed during and after therapy. Combination therapy with other myelosuppressive drugs may require modification of dosage/timing of schedules in order to minimize additive effects. Carboplatin courses should not, in general, be repeated more frequently than every 4 weeks in order to ensure that the nadir in blood counts has occurred and there has been recovery to a satisfactory level.
Patients with severe and persistent myelosuppression are at high risk of infectious complications including fatal outcomes. If any of these events occurs, carboplatin should be interrupted and dose modification or discontinuation should be considered.

Allergic reactions: As with other platinum-based drugs, allergic reactions appearing most often during administration may occur and necessitate discontinuation of infusion. Patients should be observed carefully and an appropriate symptomatic treatment (including antihistamines, adrenaline and/or glucocorticoids) must also be initiated in such cases. Cross reactions, sometimes fatal, have been reported with all the platinum compounds the vial stopper contains dry natural rubber (a derivative of latex), which may cause allergic reactions.
Renal Toxicity: In patients with impaired renal function, the effect of carboplatin on the haemotopoietic system is more pronounced and longer-acting than in patients with normal renal function. In this risk group, therapy with carboplatin must be performed with special caution.
Precautions: Carboplatin should only be administered under the supervision of a qualified physician who is experienced in the use of chemotherapeutic agents. Diagnostic and treatment facilities should be readily available for management of therapy and possible complications.
Peripheral blood counts, renal and hepatic function tests should be monitored closely. Blood counts should be performed prior to commencement of carboplatin therapy and at weekly intervals thereafter. The drug should be discontinued if abnormal depression of the bone marrow or abnormal renal or hepatic function is seen.
Haematologic Toxicity: Leukopenia, neutropenia, and thrombocytopenia are dose-dependent and dose-limiting. Peripheral blood counts should be monitored during carboplatin treatment. This will monitor toxicity and help determine the nadir and recovery of haematological parameters and assist in subsequent dosage adjustments. Median day of nadir is day 21 in patients receiving single agent carboplatin and day 15 in patients receiving carboplatin in combination with other chemotherapeutic agents. In general, single intermittent courses of carboplatin should not be repeated until leukocyte, neutrophil, and platelet counts have returned to normal. Lowest levels of platelets are generally seen between days 14 and 21 of initial therapy. A greater reduction is seen in patients who previously received extensive myelosuppressive chemotherapy. Lowest levels of white cells occur generally between days 14 and 28 of initial therapy. If neutrophil levels fall below 2000 cells/mm³ or platelets are less than 100,000 cells/mm³ then postponement of carboplatin therapy until bone marrow recovery is evident, should be considered. This recovery usually takes 5 to 6 weeks. Transfusions may be necessary and dosage reductions recommended for subsequent treatment.
Anaemia is frequent and cumulative, however rarely requires a transfusion.
Haemolytic-uraemic syndrome (HUS): Haemolytic-uraemic syndrome (HUS) is a life-threatening side effect. Carboplatin should be discontinued at the first signs of any evidence of microangiopathic haemolytic anaemia, such as rapidly falling haemoglobin with concomitant thrombocytopenia, elevation of serum bilirubin, serum creatinine, blood urea nitrogen, or LDH. Renal failure may not be reversible with discontinuation of therapy and dialysis may be required.
Renal toxicity: The incidence and severity of nephrotoxicity may increase in patients who have impaired kidney function before carboplatin treatment. It is not clear whether an appropriate hydration programme might overcome such an effect but dosage reduction or discontinuation of therapy is required in the presence of severe alteration in renal function test. Impairment of renal function is more likely in patients who have previously experienced nephrotoxicity as a result of Cisplatin therapy.
Neurologic Toxicity: Although peripheral neurologic toxicity is generally common and mild, limited to paresthesia and decreases in osteotendinous reflexes, its frequency is increased in patients older than 65 years and/or in patients previously treated with cisplatin. Monitoring and neurological examinations should be carried out at regular intervals.
Visual disturbances, including loss of vision, have been reported after the use of carboplatin in doses higher than those recommended in patients with renal impairment. Vision appears to recover totally or to a significant extent within weeks of stopping these high doses.
Reversible Posterior Leukoencephalopathy Syndrome (RPLS): Cases of Reversible Posterior Leukoencephalopathy Syndrome (RPLS) have been reported in patients receiving carboplatin in combination chemotherapy. RPLS is a rare, reversible (after treatment discontinuation), rapidly evolving neurological condition, which can include seizure, hypertension, headache, confusion, blindness, and other visual and neurological disturbances. Diagnosis of RPLS is based upon confirmation by brain imaging, preferably MRI (Magnetic Resonance Imaging).
Geriatric Use: In studies involving combination therapy with carboplatin and cyclophosphamide, elderly patients treated with carboplatin were more likely to develop severe thrombocytopenia than younger patients. Because renal function is often decreased in the elderly, renal function should be considered when determining dosage.
Other: Auditory defects have been reported during carboplatin therapy. Ototoxicity may be more pronounced in children and is more likely seen in patients previously treated with cisplatin. Cases of hearing loss with a delayed onset have been reported in pediatric patients. A long-term audiometric follow-up in this population is recommended.
Administration of live or live-attenuated vaccines in patients immunocompromised by chemotherapeutic agents including carboplatin may result in serious or fatal infections. Vaccination with a live vaccine should be avoided in patients receiving carboplatin. Killed or inactivated vaccines may be administered; however, the response to such vaccines may be diminished. Aluminium containing equipment should not be used during preparation and administration of Carboplatin.

A12 INTERACTIONS WITH OTHER MEDICAMENTS
Carboplatin may interact with aluminium to form a black precipitate. Needles, syringes, catheters or IV administration sets that contain aluminium parts which may come into contact with carboplatin, should not be used for the preparation or administration of the drug.
Due to the increase of thrombotic risk in cases of tumoral diseases, the use of anticoagulative treatment is frequent. The high intra-individual variability of the coagulability during diseases, and the possibility of interaction between oral anticoagulants and anticancer chemotherapy, may require an increase in frequency of INR monitoring if a patient is treated with oral anticoagulants.
Concomitant use contraindicated: Yellow fever vaccine: risk of generalized disease mortal.
Concomitant use not recommended:
- Live attenuated vaccines (except yellow fever): Risk of systemic, possible fatal disease. This is increased in patients who are already immunosuppressed by their underlying disease. Use inactivated vaccine where this exist (poliomyelitis).
- Phenytin, fosphenytoin: Risk of exacerbation of convulsions (resulting from the decrease of phenytoin digestive absorption by the cytotoxic drug), risk of toxicity enhancement or loss of efficacy of the cytotoxic drug (due to increased hepatic metabolism by phenytoin).
Concomitant use to take into consideration:
- Ciclosporin (and by extrapolation tacrolimus and sirolimus): Excessive immunosuppression with risk of lymphoproliferation.
- Concurrent therapy with nephrotoxic drugs or ototoxic drugs such as amino glycosides, vancomycin, capreomycin and diuretics, may increase or exacerbate toxicity, particularly in renal failure patients, due to Carboplatin induced changes in renal clearance.
- Loop diuretics: The concomitant use of carboplatin with loop diuretic should be approached with caution due to the cumulative nephrotoxicity and ototoxicity.
Combination therapy with other myelosuppressive agents may require dose changes or rescheduling of doses in order to minimize the additive myelosuppressive effects.
A13 PREGNANCY AND LACTATION
Pregnancy: Carboplatin can cause foetal harm when administered to a pregnant woman. Carboplatin has been shown to be embryotoxic and teratogenic in rats receiving the drug during organogenesis. No controlled studies in pregnant women have been conducted. Safe use of carboplatin in pregnancy has not been established. Both men and women receiving carboplatin should be informed of the potential risk of adverse effects on reproduction.
Women of childbearing potential should be advised to avoid becoming pregnant by using effective contraception and should be fully informed of the potential hazard to the foetus should they become pregnant during carboplatin therapy.

Carboplatin should not be used in pregnant women or women of childbearing potential who might become pregnant unless the potential benefits to the mother outweigh the possible risks to the foetus.
Breastfeeding: It is not known whether Carboplatin is excreted in breast milk. To avoid possible harmful effects in the infant, breast-feeding must be stopped during carboplatin therapy.
Fertility: Gonadal suppression resulting in amenorrhoea or azospermia may occur in patients receiving antineoplastic therapy. These effects appear to be related to dose and length of therapy and may be irreversible. Prediction of the degree of testicular or ovarian functional impairment is complicated by the common use of combinations of several antineoplastic, which makes it difficult to assess the effects of individual agents.
Men of sexually mature age treated with carboplatin are advised not to father a child during treatment and up to 6 months afterwards. Male patients should seek advice about sperm preservation prior to initiation of the therapy because of the possibility of irreversible infertility due to therapy with carboplatin.

System Organ Class	Frequency	MedDRA Term
Neoplasms, benign and malignant and unspecified (incl cysts and polyps)	Not Known	Treatment related secondary malignancy
Infections and infestations	Common	Infections*
	Not known	Pneumonia
Blood and lymphatic system disorders	Very Common	Thrombocytopenia, neutropenia, leukopenia, anaemia
	Common	Haemorrhage*
	Not Known	Bone marrow failure, febrile neutropenia, haemolytic uraemic syndrome
Immune system disorders	Common	Hypersensitivity, anaphylactoid type reaction
Metabolism and nutrition disorders	Not known	Dehydration, anorexia, Tumour lysis syndrome, hyponatraemia
Nervous system disorders	Common	Neuropathy peripheral, paraesthesia, decrease of osteotendinous reflexes, sensory disturbance dysgeusia
	Not known	Cerebrovascular accident*, encephalopathy, Reversible Posterior Leukoencephalopathy Syndrome (RPLS)
Eye disorders	Common	Visual disturbance (incl. rare cases of loss of vision)
Ear and labyrinth disorders	Common	Otitotoxicity
Cardiac disorders	Common	Cardiomyocardial disorder*
	Not known	Cardiac failure*
Vascular disorders	Not known	Embolism*, hypertension, hypotension
Respiratory, thoracic and mediastinal disorders	Common	Respiratory disorder, interstitial Lung disease, bronchospasm
Gastrointestinal disorders	Very common	Vomiting, nausea, abdominal pain
	Common	Diarrhoea, constipation, mucous membrane disorder
	Not known	Stomatitis, pancreatitis
Skin and subcutaneous tissue disorders	Common	Alopecia, skin disorder
	Not known	Urticaria, rash, erythema, pruritus
Musculoskeletal and connective tissue disorders	Common	Musculoskeletal disorder
Renal and urinary disorders	Common	Urogenital disorder
General disorders and administration site conditions	Common	Asthenia
	Not known	Injection site necrosis, injection site reaction, injection site extravasation, injection site erythema, malaise
Investigations	Very Common	Creatinine renal clearance decreased, blood urea increased, blood alkaline phosphatase increased, aspartate amino transferase increased, liver function test abnormal, blood sodium decreased, blood potassium decreased, blood calcium decreased, blood magnesium decreased
	Common	Blood bilirubin increased, blood creatinine increased, blood uric acid increased

*Fatal in <1%, fatal cardiovascular events in <1% included cardiac failure, embolism, and cerebrovascular accident combined.
Blood and lymphatic system disorders: Myelosuppression is the dose-limiting toxicity of carboplatin injection. In patients with normal baseline values, thrombocytopenia with platelet counts below 50,000/mm³ occurs in 25% of patients, neutropenia with granulocyte counts below 1,000/mm³ in 18% of patients, and leukopenia with WBC counts below 2,000/mm³ in 14% of patients. The nadir usually occurs on day 21. Myelosuppression can be worsened by combination of carboplatin injection with other myelosuppressive compounds or forms of treatment.
Myelotoxicity is more severe in previously treated patients, in particular in patients previously treated with cisplatin and in patients with impaired kidney function. Patients with poor performance status have also experienced increased leukopenia and thrombocytopenia. These effects, although usually reversible, have resulted in infectious and hemorrhagic complications in 4% and 5% of patients given carboplatin injection, respectively. These complications have led to death in less than 1% of patients. Anaemia with haemoglobin values below 8g/dl has been observed in 15% of patients with normal baseline values. The incidence of anaemia is increased with increasing exposure to carboplatin injection.
Neoplasms benign, malignant and unspecified (including cysts and polyps): Secondary acute malignancies after cytostatic combination therapies containing carboplatin have been reported.
Respiratory, thoracic and mediastinal disorders: Pulmonary fibrosis manifested by tightness of the chest and dyspnoea. This should be considered if a pulmonary hypersensitivity state is excluded (see General disorders below).
Gastrointestinal disorders: Vomiting occurs in 65% of patients, in one-third of whom it is severe. Nausea occurs in an additional 15%. Previously treated patients (in particular patients previously treated with cisplatin) appear to be more prone to vomiting. Nausea and vomiting are generally delayed until 6 to 12 hours after administration of carboplatin, are readily controlled or prevented with antiemetics and disappear within 24hours. Vomiting is more likely when carboplatin injection is given in combination with other emetogenic compounds. The other gastrointestinal complaints corresponded to pain in 8% of patients, diarrhoea, and constipation in 6% of patients. Cramps have also been reported.
Nervous system disorders: Peripheral neuropathy (mainly paresthesias and decrease of osteotendinous reflexes) has occurred in 4% of patients administered carboplatin injection. Patients older than 65 years and patients previously treated with cisplatin, as well as those receiving prolonged treatment with carboplatin injection, appear to be at increased risk.
Clinically significant-sensory disturbances (ie, visual disturbances and taste

modifications) have occurred in 1% of patients.
The overall frequency of neurologic side effects seems to be increased in patients receiving carboplatin injection in combination. This may also be related to longer cumulative exposure. Paresthesias present prior to treatment, especially if caused by cisplatin, may persist or worsen during carboplatin therapy.
Eye disorders: Visual disturbances, including sight loss, are usually associated with high dose therapy in renally impaired patients.
Ear and labyrinth disorders: A subclinical decrease in hearing acuity in the high frequency range (4000-8000 Hz), determined by audiogram, occurred in 15% of patients. Very rare cases of hypacusia have been reported.
Tinnitus was also commonly reported. Hearing loss as a result of cisplatin therapy may give rise to persistent or worsening symptoms. At higher than recommended doses, in common with other ototoxic agents, clinically significant hearing loss has been reported to occur in paediatric patients when carboplatin is administered.
Hepatobiliary disorders: Modification of liver function in patients with normal baseline values was observed, including elevation of total bilirubin in 5%, SGOT in 15%, and alkaline phosphatase in 24% of patients. These modifications were generally mild and reversible in about one-half the patients.
In a limited series of patients receiving very high doses of carboplatin injection and autologous bone marrow transplantation, severe elevation of liver function tests has occurred. Cases of an acute, fulminant liver cell necrosis occurred after high-dose administration of carboplatin.
Renal and urinary disorders: When given in usual doses, development of abnormal renal function has been uncommon, despite the fact that carboplatin injection has been administered without high-volume fluid hydration and/or forced diuresis. Elevation of serum creatinine occurs in 6% of patients, elevation of blood urea nitrogen in 14%, and of uric acid in 5% of patients. These are usually mild and are reversible in about one-half the patients. Creatinine clearance has proven to be the most sensitive renal function measure in patients receiving carboplatin injection. Twenty-seven percent (27%) of patients who have a baseline value of 60 mL/min or greater, experience a reduction in creatinine clearance during carboplatin injection therapy. Impairment of renal function is more likely in patients who have previously experienced nephrotoxicity as a result of cisplatin therapy.
Immune system disorders: Anaphylactoid-type reactions, sometimes fatal, may occur in the minutes following injection of the product: facial oedema, dyspnoea, tachycardia, low blood pressure, urticaria, anaphylactic shock, bronchospasm. Fever with no apparent cause has also been reported.
Skin and subcutaneous tissue disorders: Erythematous rash, fever and pruritis have been observed. These were reactions similar to those seen after cisplatin therapy but in a few cases no cross-reactivity was present.
Investigations: Decreases in serum sodium, potassium, calcium, and magnesium occur in 29%, 20%, 22%, and 29% of patients, respectively. In particular, cases of early hyponatraemia have been reported. The electrolyte losses are minor and mostly take a course without any clinical symptoms.
Cardiac disorders: Isolated cases of cardiovascular incidents (cardiac insufficiency, embolism) as well as isolated cases of cerebrovascular accidents have been reported.
General disorders and administration site conditions: Reactions at the site of injection (burning, pain, reddening, swelling, urticaria, necrosis in connection with extravasation) have been reported.
Fever, chills and mucositis have occasionally been observed.
Reporting of suspected adverse reactions: Reporting suspected adverse reactions after authorization of the medicinal product is important. It allows continued monitoring of the benefit/risk balance of the medicinal product. Healthcare professionals are asked to report any suspected adverse reactions via National Reporting System.
A15 SYMPTOMS AND TREATMENT OF OVERDOSE
There is no known antidote for carboplatin overdose.
If necessary, however, the patient may need supportive treatment relating to myelosuppression, renal, hepatic and auditory function impairment. Reports of doses up to 1600 mg/m² indicate patients feeling extremely ill with diarrhoea and alopecia developing. Use of higher than recommended doses of carboplatin has been associated with loss of vision.
A16 EFFECTS ON ABILITY TO DRIVE AND USE MACHINE
No studies of the effects on the ability to drive and use machines have been performed. However, carboplatin may cause nausea, vomiting, vision abnormalities and ototoxicity; therefore, patients should be warned of the potential effect of these events on the ability to drive or to use machines.
A17 PRECLINICAL SAFETY DATA
Not Applicable
A18 INSTRUCTIONS FOR USE
Carboplatin is given as a single short term intravenous infusion over 15 to 60 minutes.
A19 STORAGE CONDITION
Do not store above 30 °C. Protect from light.
Keep vial in outer carton in order to protect from light.
For single use. Discard unused portion.
A20 SHELF LIFE
Unopened: 2 Years
After dilution (in-use): Carboplatin solution for infusion can be further diluted to concentration as low as 0.5 mg/ml with 5% Dextrose or 0.9 % Sodium Chloride Injection. The infusion solution is chemically stable for up to 24 hours when stored at 2-8 °C and up to 8 hours when stored at 25 °C.
From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.
A21 THERAPEUTIC CODE
ATC code: L01XA02
INCOMPATIBILITIES
This medicinal product must not be mixed with other medicinal product except those mentioned in section A20.
NATURE & CONTENT OF CONTAINER
Pack size- 15ml
CARBOL (Carboplatin Concentrate for Solution for Infusion 10 mg/ml) is supplied in a 20 ml moulded clear glass vial with reinforced Non PVC base packed in unit carton along with package insert.
Pack size- 45ml
CARBOL (Carboplatin Concentrate for Solution for Infusion 10 mg/ml) is supplied in a 50 ml moulded clear glass vial with reinforced Non PVC base packed in unit carton along with package insert.
HANDLING AND DISPOSAL
Caution should be exercised in handling and preparing Carboplatin Injection. To minimize the risk of dermal exposure, always wear impervious gloves when handling vials containing Carboplatin Injection. If Carboplatin Injection contacts the skin, immediately wash the skin thoroughly with soap and water. If Carboplatin Injection contacts mucous membranes, the membranes should be flushed immediately and thoroughly with water.

Manufactured by: **VENUS REMEDIES LIMITED**
Hill Top Industrial Estate, Jharmajin, EP1P
Phase-I (Extn.), Bhatoli Kalan, Baddi,
Distt. Solan, Himachal Pradesh, 173205, India
Product Registration Holder: **Unimed Sdn Bhd**
53, Jalan Tembaga SD 5/2B, Bandar Sri
Damansara, 52200, Kuala Lumpur, Malaysia
Date of revision of package insert: 05/02/2022

VENUS REMEDIES LIMITED

Product Name : Carbol 150 mg & 420 mg

Party Name : Malaysia

Size : 170 x 300 mm

Colours : BLACK

Paper Specification Code : 60 GSM Maplitho (with fold)

Date : 06-05-2020, 11.06.21, 26-08-2021, 30-12-2021, 27-01-2022, 05-02-2022

2-D Barcode : Not Applicable

Artwork Code : Tentative Artwork

Embossing : No

Brail : No

Other Process : No

Foil Colour : No

Foil Stamp : No

Plate Changes : New Artwork

Reference : Size refer from Oman Tender

Grain :